Blast Injury / Incident

TB-01 | Trauma & Burns

Quick flow

01 Scene Safe? [EMT / AEMT / Paramedic]

Stage for hazards, structural collapse, fire, hazardous material, or secondary device; activate the agency MCI plan when indicated.

02 Triage + Rapid Removal [EMT / AEMT / Paramedic]

Quantify patients, request resources, apply the local MCI/triage process when indicated, and begin load-and-go care.

03 Identify Injury Pattern [EMT / AEMT / Paramedic]

Airway burn • blast lung • hemorrhage • penetrating trauma • crush; consider radiation only when the mechanism suggests a radiological or nuclear event.

04 Treat Immediate Threats [EMT / AEMT / Paramedic]

Airway and ventilation • hemorrhage control • assess chest injury • hypothermia prevention • reassessed shock care. Urgently request ALS for suspected tension pneumothorax; decompression is Paramedic-only. Ventilate when clinically necessary.

05 Blast Lung Injury? [EMT / AEMT / Paramedic]

Dyspnea, hypoxia, hemoptysis, wheeze, chest pain, or diminished breath sounds; perform serial respiratory assessment, SpO_2, lung sounds, and monitoring when available.

06 Rapid Trauma Transport [EMT / AEMT / Paramedic]

Use trauma/burn destination plan and notify early, especially for respiratory compromise, penetrating injury, major burn, crush injury, or multisystem trauma.

Overview

- Treat every blast as a potentially unsafe, intentional event until authorities determine otherwise.
- Expect combined primary, secondary, and tertiary blast injury, including barotrauma, penetrating trauma, burns, crush injury, and toxic exposure.

Provider-Level Actions

Perform all immediately indicated care within scope without delaying rapid trauma transport.

EMT

- Maintain scene awareness, perform triage, control major hemorrhage, assess chest injury, oxygenation, ventilation, and perfusion, urgently request ALS for suspected tension pneumothorax, support ventilation, and prevent hypothermia.
- For an open chest wound in a spontaneously breathing patient, consider an approved vented seal and reassess frequently. If deterioration follows sealing, assess/release an obstructing seal and obtain immediate Paramedic treatment for suspected tension pneumothorax. Do not probe the wound. Positive-pressure ventilation requires the chest-injury standing orders and immediate Paramedic/Medical Control guidance, not automatic extrapolation of this seal instruction.
- Assess for hearing loss, ocular injury, burns, penetrating injury, abdominal injury, amputation, and crush syndrome.

AEMT

- Perform all EMT care plus IV access and reassessed crystalloid only when indicated for shock after hemorrhage control, without delaying transport.

- Use authorized airway adjuncts and monitor closely for worsening blast-lung physiology.

Paramedic

- Perform all prior care plus advanced airway management, cardiac/$EtCO_2$ monitoring, and chest decompression when indicated.
- When positive-pressure ventilation is clinically necessary for suspected blast lung injury, use the lowest effective pressures/volumes and monitor closely for deterioration.

Indications

- Patient with suspected blast injury / incident requiring prehospital assessment or treatment.

Warnings / precautions

- Do not approach a patient who may be attached to wires, a package, or a triggering mechanism; withdraw and notify law enforcement.
- Do not withhold needed ventilation. Avoid unnecessarily high positive pressures/volumes and excessive fluids in suspected blast lung injury; if air transport is used, notify the flight crew of the concern.

Clinical pearls

- Tympanic membrane rupture and hearing loss are common and may impair communication.
- Secondary blast injuries from debris and shrapnel are the most common cause of death.

Source and references

Current Tennessee EMS scope of practice, verified individual credentials, and Claiborne County standing orders control provider authorization.

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